



NZSA

Relative Value Guide

2021

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Relative Value Guide (NZSA RVG)

SECTION I

Preamble

Anaesthesia is the practice of medicine that provides services that include, but is not limited to:

- The management of a patient to render them insensible to pain and emotional stress during surgical, obstetric and medical procedures
- The support of physiological stability under the stress of anaesthetic and surgical manipulations
- The clinical management of the unconscious patient
- The management of pain relief
- The management of cardiac and respiratory resuscitation
- The application and management of respiratory therapy
- The clinical management of fluid, electrolyte and metabolic disturbances
- Peri-operative care of the patient before, during and after surgery including professional attendances and consultations in relation to all of the above.

Purpose of the NZSA Relative Value Guide

The NZSA RVG recognises this peri-operative role of the Anaesthetist, which can be an extended period beyond hospital discharge.

The NZSA RVG is a NOT a fee schedule; it is simply one method of assessing units spent in relation to specific anaesthetic services, which can aid in calculating anaesthesia fees. The NZSA RVG can be a useful guide, however anaesthetists are not compelled to use it.

The NZSA RVG relates anaesthesia services, one to the other, on the basis of the complexity of the service and the time to undertake that service. It is a system that expresses relativities in terms of units and is designed to assist anaesthetists to develop their own reimbursement schedules in a logical and consistent manner.

The guide does not suggest the monetary value of any unit or the dollar amount of any individual anaesthetist's charges. Reimbursement based on the RVG is derived by multiplying the total relative value units by a monetary value.

In New Zealand, private practice anaesthetists mostly work as individual practitioners. Accordingly, as required by the Commerce Act (1986), each individual anaesthetist must choose their monetary value independently. More detail relating to this can be found on the Commerce Commission website: <https://comcom.govt.nz/business>

Individual anaesthetists are entitled to be aware of the "market rate" charged by anaesthetists for a unit value. However, they should not have an arrangement or understanding with other anaesthetists that they will price at that "market rate."

Anaesthetists working in partnerships, joint venture arrangements or company structures should seek legal advice regarding the derivation of reimbursement for each of these commercial arrangements.

Hospitals and surgeons may hold contracts such as the Elective Services Contract, Affiliated Provider Contracts and DHB contracts, to name some of the many contracts in place. As a subcontractor, an anaesthetist should discuss the derivation of their contract reimbursement with the contract holder.

If the individual works independently this should be undertaken on an individual basis. Legal advice should be sought before such arrangements are entered into and regarding arrangements that would allow any form of collective negotiation.

Fixed Contract Reimbursement

All discussions about the level of reimbursement for anaesthetic services under these contracts should be held between the contract holder and the individual anaesthetist, or representative, if the anaesthetist works within a legal entity that enables collective negotiations. If you feel that the fixed fee applied to your patient is unreasonably low, due to patient complexity, then it is reasonable to request a 'top up' which may utilise the RVG modifiers as a guide. For example, this might be +2 units for morbid obesity, + 2 time units for an otherwise simple tooth extraction. The critical step is discussing this with the contract holder/insurance provider prior to service provision.

As a subcontractor it is important for you to understand your obligations under the Lead Contract and it is recommended that you ask to view the relevant anaesthetic sections of any contract. Many hospitals produce Contracts for Anaesthetic Services – be aware of these and their contents so you understand the conditions, your obligations and your reimbursement for the provision of anaesthetic services.

Additional Anaesthetic Assistance

Where it is known that two anaesthetists are needed for a case, whatever the circumstances, prior approval must be sought from the Lead Provider of that contract before surgery to ensure appropriate remuneration.

Informed Financial Consent

The NZSA strongly recommends that in order to fulfil HDC requirements of informed consent, each practitioner should discuss their fees with patients

prior to their admission to hospital, or prior to the provision of services. A patient's right to be fully informed (Right 6) and to make informed choice and give informed consent (Right 7) include the financial impact, e.g. having to pay a gap between the insurer's reimbursement and the total anaesthesia fee. Refer to www.hdc.org.nz for more information.

The 'gold standard' for informed financial consent is a written estimate of the anaesthesia fee (a range is acceptable) along with a reasonable indication of expenses that would be additional to that reimbursed by insurers (out-of-pocket expenses, or gap).

Obtaining financial consent is more difficult for anaesthetists than most other specialists, particularly for day surgery patients, day of surgery admissions or emergency admissions.

The following list describes some of the methods than can be used to obtain financial consent:

- See your patients in consulting rooms before their admission to hospital. In this ideal situation verbal and written consent can be obtained.
- Ask your surgeon/s to encourage patients who you will be anaesthetising to contact you or your rooms/secretary for an estimate of anaesthetic fees.
- Have your rooms/secretary contact all your patients, prior to admission, with an estimate.
- Inform your surgeons and/or their secretaries of your approximate fees and ask them to inform your patients. It should be made clear that any queries are to be directed to you or your rooms/secretary.
- Provide your surgeons and/or their secretaries with a n information hand-out about your fees.
- Inform your patients of your fees when you see them for their pre-anaesthetic consultation.
- If you are unable to inform patients of your fees prior to anaesthesia, do so as soon as practical post-operatively.

The following is an example of a document to provide to your patient with clarification of the estimate for anaesthesia fees where an out-of-pocket expense is expected.

Estimate of Medical Fee - Anaesthesia

The following estimate indicates the likely costs you will be required to pay for the anaesthesia services associated with your operation or procedure.

Please note that this is an estimate only. Unless specified, the estimate refers only to the fees charged by this anaesthetist. It does not cover services provided by other doctors, or other costs associated with your treatment. Sometimes, in the event of unforeseen circumstances, it may be necessary to arrange additional medical services resulting in further charges to you.

If you have any concerns or require further information, please discuss this with your anaesthetist or anaesthetist's secretary, preferably before your procedure.

Patient Name: _____

Hospital: _____

Health Fund Name: _____

Proposed Procedure: _____

Estimated Time of Procedure: _____ Date of Admission: _____

Description of Service (s)	Fee	Estimated Total Benefit (see below)*	Estimated Gap Payment (see below)**

*Estimated total Benefit - this is the amount your insurer will pay. For further information, please contact your insurer.

** Estimated Gap Payment - this is the amount that is not covered by your insurer and will be charged to you.

Derivation of Relative Value Units

RVG based reimbursements are calculated using:

- PROCEDURAL/BASE UNITS
- TIME UNITS and
- MODIFYING UNITS.

PROCEDURE/BASE UNITS

Base units are categorised according to the anatomical site of the procedure being performed, the patient position and the complexity of the anaesthesia required. Base units take into account the anticipated physiological stressors of the given procedure.

Patient complexity separate to the surgical procedure comes under modifying units.

The base units take into account a brief (<15 minutes) pre and post-operative visit.

When multiple procedures are performed during one anaesthetic only ONE Base Unit should be charged according to the RVG. If a second procedure requires a change of patient position, modifying units may need to be added.

TIME UNITS

The NZSA recognises the role of the anaesthetist in the perioperative period, not just the time of surgery.

Anaesthetic time commences when the anaesthetist begins exclusive and continuous care of the patient and ceases when the anaesthetist is no longer in professional attendance i.e. when the patient is safely placed under the supervision of other personnel. This includes time spent before and after surgery e.g. time related to straightforward consent, the insertion of blocks and time spent involved in PACU care post-surgery.

Contract issues relating to start and finish times for anaesthesia need to be discussed by the individual anaesthetist or contracting arrangement e.g. joint venture/company/partnership with the Lead Provider of the contract.

MODIFYING UNITS

Modifying units are to be used to recognise the time commitment involved in pre-operative and post-operative care. They should also be used to reflect the increase in intraoperative care that is required when patient co-morbidity or complexity is significant, e.g.

- ASA score, e.g. >2
- Extremes of age, e.g. <1, 1-2, 70-80, >80
- Elevated BMI, e.g. >35
- Frailty
- Emergency operation.

The NZSA recognises that unexpected intraoperative crises do occur and no single RVG code has been generated to reflect this. However, should a crisis occur, the added time units, and medical transport code if applicable can reflect the increase in care and we consider it reasonable to add modifiers for the resuscitation and insertion of invasive monitoring, e.g. 2 units for an arterial line, 2 units for central line.

Summary of Changes from 2015 RVG

- Grouping the operations by anatomical site
- Simplification of display of modifying units
- Increasing graduated modifying units for extremes of age and morbid obesity
- Increasing base units for HDU cover from home to 4-6 units depending on patient complexity
- Example of informed financial consent
- More detail of pre-operative assessment on ACC patients
- Updated links and further information.

SECTION II

Base units for standard procedures

CODE	PROCEDURE	UNITS
	Supervision	
SP 1	Provision of IV Sedation and/or Monitoring	4
	Head	
H1	Minor Procedures e.g. Excision of Superficial Lesions, Myringotomies	4
H2	Simple but Invasive Procedures e.g. Adenoidectomy, Tonsillectomy, Myringoplasty, Ear canal, Septoplasty	5
H3	Moderate Procedures e.g. Endoscopic Sinus Surgery, Septo-rhinoplasty, Micro-laryngoscopy, Bronchoscopy, Parotidectomy, Face-lift, Orbital Reconstruction	6
H4	Complex Procedures e.g. Maxillary/Mandibular Osteotomy, Tumour Resection, Free Flap Placement	10-12
Dental		
H5	Dental Extractions/Dental Restorations	5
Ocular		
H6a	Ocular Procedure Under Local Anaesthetic or Eye Block or GA > 2yo	4
H6b	Ocular Procedure Under GA [YOUNGER than Aged TWO Years]	5
Neurosurgery	*These Base Units include Loading for Prone Positioning if needed*	(+2)
H7	Cranioplasty/Cerebrospinal fluid shunt procedures supine/lateral	
A	Supine/lateral	10
b	Prone/park bench/sitting	12
H8	Non-Vascular Open Intracranial/Spinal cord	
a	Supine/lateral	12
b	Prone/park bench/sitting	14
H9	Vascular Open Intracranial/Spinal cord Procedures	
a	Supine/lateral	20
b	Prone/park bench/sitting	22
	Neck (Non vascular)	
N1	Minor Procedures e.g. Excision of Superficial Lesions	4
N2	Simple but Invasive Procedures e.g. Closure of Tracheostomy	5
N3	Moderate Procedures e.g. Thyroidectomy, Simple Neck Dissection (Not part of more extensive surgery)	6
N4	Significant Complex Procedures e.g. Laryngectomy, Carotid Endarterectomy, Tumour Resection +/- Neck Dissection, Free Flap Placement	10-14

	UPPER LIMB	
UL1	Minor Procedures e.g. Excision Skin Lesions, Carpal Tunnel Release	4
UL2	Simple but Invasive Procedures e.g., Simple Tendon Transposition, Simple Nerve Transposition, Open Radius/Ulnar Surgery, straightforward removal of metal wear	5
UL3	Moderate Soft Tissue/Muscle/Nerve Procedures e.g. Rotator Cuff Surgery, Wrist Joint Replacement, Finger Joint Replacement/Fusion, Wrist Arthroscopy, complicated removal of metal wear	6
UL4	Major Bony/Joint Procedures e.g. Shoulder Replacement arthroplasty upper limb	8
	<i>*Add 2 units if sitting position*</i>	+2
	THORAX	
T1	Minor Procedures e.g. Excision of Superficial Lesions (Shoulder/Chest/Back)	4
Breast		
T2	Minor Procedures e.g. Removal Simple Breast Lesion	4
T2	Simple but Invasive Procedures e.g., Wide Local Excision Breast Lump, Breast Augmentation, Breast Reduction, Simple Mastectomy	5
T3	Moderate Procedures e.g. Complex Mastectomy (includes axillary node surgery) dissection	6
T4	Major Procedures e.g. TRAM Flap	8
Thoracic		
T5a	Mediastinoscopy	6
T5b	Sympathectomy Lobectomy/Wedge Resection	8
T5c	Pleurodesis/Simple Biopsy	8-10
T5d	Lobectomy/Wedge Resection	14
T5e	Major Thoracic e.g. Pneumonectomy, Tracheal and/or Bronchial Reconstruction, Thoracoabdominal Procedures	20
Cardiac		
T6a	Sternal Debridement or Chest Re-Opening (either OR or ICU)	8
T6b	Less Complex Surgery e.g. CABG Only, Single Valve Replacement	16
T6c	More Complex Surgery e.g. Redo Case, Multiple Valve Replacement, CABG Plus Valve	20
T6d	Percutaneous Cardiac Surgery e.g. TAVI (NOT EP Ablations)	15
TOE Use		
T7a	Insertion TOE + Limited Exam e.g. TOE During EP Procedures at Request of Cardiologist	2
T7b	Diagnostic TOE: Full Examination, Including Report [Nil Time Units]	6

	SPINE	
	<i>These Base Units include loading for prone positioning if needed</i>	
S1	Removal of Spinal Hardware	8
Cervical		
S2	Anterior Cervical Procedures e.g. Anterior Disc Fusion, Disc Replacement	8
S3a	Posterior Cervical Procedures [1-2 levels] e.g. Foraminotomy	10
S3b	Posterior Cervical Procedures [>2 levels or Front + Back]	12
Thoracic		
S4	Intrathoracic Spinal Procedures	16
S5	Extensive Thoracic/Lumbar Decompression and Fusion e.g. Scoliosis	20
Lumbar		
S6	Discectomy/Decompression Procedures – [1-2 levels]	8
S7	Minimally Invasive Lateral Lumbar Procedures	8
S8a	Anterior Decompression/Fusion – Single Level	8
S8b	Anterior Decompression/Fusion – Multiple Level	10
S9a	Posterior Decompression/Fusion – Single Level	10
S9b	Posterior Decompression/Fusion – Two Level	12
S9c	Posterior Decompression/Fusion – Three + Level	14
S10	Combined Anterior and Posterior Surgical Approach	18
	ABDOMEN	
A1	Minor Procedures e.g. Excision Of Skin Lesions, Egg Pick-up (Sedation)	4
A2	Simple but Invasive Procedures e.g. Hernias (Open + Laparoscopic), Gynae: Diagnostic Laparoscopy, Tubal Ligation, Ovarian Egg Pick-up (GA)	5
A3	Moderate Procedures e.g. Incisional Hernia Repair, Laparoscopic Cholecystectomy, Appendectomy, Ectopic Pregnancy	6
A4	Gynae: Moderate/Major Procedures e.g. Endometriosis Surgery, Abdominal Hysterectomy (Open/Laparoscopic), Ovarian Surgery	6-8
A5	Major Procedures e.g. Extensive Incisional Hernia Repair, Gynaecological Tumour Surgery, Extensive Liposuction, Abdominoplasty	8
A6	Complex Procedures e.g. Pheochromocytoma	12-14
Upper GI		
A7	Major Laparoscopic Intraperitoneal Procedures e.g. Nissen Fundoplication, Gastric Banding, Sleeve Gastrectomy	8
A8	Major Open Procedure e.g. Gastric Bypass	10
A9	Complex Procedures e.g. Liver Resection, Whipple's	16
A10	Oesophagectomy	20
Lower GI		
A11	Moderate Procedures e.g. Reversal Loop Ileostomy	6

A12	Major Procedures e.g. Bowel Resection [Single Section with Re-anastomosis] INCLUDES Open/Laparoscopic/Combined Technique	8-10
A13	Complex Procedures e.g. Extensive Bowel Resection – Extending into Upper Abdomen or Pelvis [APR], AND/OR Where an – ostomy is created Open Abdominal Urological Procedures, Cystectomy	12-14
PERINEUM		
P1	Minor Procedures e.g. Excision Superficial Lesions, Excision Skin Tags, Banding Haemorrhoids, Circumcision (> 2 y.o.) Gynae: Cystoscopy, Hysteroscopy, D+C, Labiaplasty	4
P2	Simple but Invasive Procedures e.g. Circumcision (< 2 y.o.), Orchidopexy, Scrotal Surgery, Urethroplasty, Ureteroscopy, TURBT Gynae: Anterior Repair, Posterior Repair, Sacrospinous Fixation	5
Urology		
P3	Minor Procedures e.g. Trans-perineal Prostate Procedures such as Rod Insertion or Prostate Biopsy	4
P4	Moderate Surgery e.g. TURP	6
P5	Major Surgery e.g. Percutaneous Nephrolithotomy	8
P6	Complex Surgery: Prostatectomy (Radical/Robotic), Nephrectomy, Open Cystectomy	10
Obstetric		
P6	Obstetric Vaginal Procedures	4
P7	Obstetric Epidural Pain Relief/Evacuation Of Uterus [Post-partum]	5
P8	Caesarean Section (Spinal OR GA), Vaginal Birth under GA	6
PELVIC		
PE1	Minor Procedures e.g. Application Hip Spica	4
PE2	Significant Procedures e.g. Open Pelvic Surgery	12
VASCULAR		
V1	Minor Procedure, e.g. Varicose Vein Surgery, Port Insertion	4
V2	Moderate Procedure, e.g. Infra-inguinal Arterial Surgery, Fistula Formation, other Non-aortic or Carotid Vascular Procedure	6
V3	Major Procedure, e.g. Fem-pop Bypass, Non-carotid Vascular Head and Neck Procedure	8
V4	Significant Procedure, e.g. Carotid Endarterectomy, Percutaneous Abdominal Vascular Procedure	10
V5	Complex Procedure, e.g. Open Abdominal Vascular Procedure	20
LOWER LIMB		
LL1	Minor Procedures e.g. Excision Skin Lesions, Hip Arthroscopy, Knee Arthroscopy	4
LL2	Simple but Invasive Procedures e.g. Simple Tendon Transposition, Simple Nerve Transposition	5

LL3	Moderate Soft Tissue/Muscle/Nerve Procedures e.g. ACL Surgery, Femoral/Tibial Osteotomy, Alignment, Corrective Foot Surgery	6
LL4	Major Hip/Knee Joint Procedures e.g. Hip Replacement, Knee Replacement	8
LL5	Complex Hip/Knee Joint Procedures e.g. Bilateral Joint Replacements, Simple Joint Revision	10
LL6	Significant Hip/Knee Joint Revision Procedures e.g. Two Component Revisions, Acetabular Reconstruction + Bone Graft, Open Pelvic Surgery	12
ANAESTHESIA IN REMOTE LOCATIONS		
Endoscopy		
E1	Endoscopy Under Sedation [Gastroscopy, Colonoscopy - includes Sedation for Uncomplicated Biopsies]	4
E2	Endoscopy Under General Anaesthesia [Gastroscopy, Colonoscopy]	5
E3	Moderate Endoscopic Procedures e.g. ERCP, Oesophageal Stenting	6
Cardiology		
C1	Minor Procedures e.g. Cardioversion, Sedation For TOE	4
C2	Cardiac Ablation - Right Heart, SVT Procedures	6
C3	Pulmonary Vein Isolation	10
C4	Extraction Of Cardiac Leads	12
Radiology		
R1	Diagnostic Radiology e.g., CT Scan, PET Scan, MRI	5
R2	Major Radiological Procedures e.g. Non-Cerebral Embolization, Non-Aortic Stenting	6-8
R3	Significant Radiological Procedures e.g. Cerebral Embolization, Aortic Stenting	10
Miscellaneous		
M1	Minor Procedures e.g. Electroconvulsive Therapy	4
M2	Extracorporeal Shockwave Lithotripsy	6
M3	Medical Transport - Preparation, Monitoring, Ventilator, Circulatory Support	4 + Time
PAIN CONSULTATIONS AND PROCEDURES		
Consultation	For Chronic Pain Requiring History, Examination, and Decision Making	
CP1	LOW Complexity Decision Making	4 + Time
CP2	MEDIUM Complexity Decision Making	6 + Time
CP3	HIGH Complexity Decision Making	8 + Time
Procedures		
CP4	Trigger Point Injections/Infusions	4 + Time
CP5	Regional Blocks	6 + Time
CP6	Spinal Procedures	8 + Time
<i>NB: If multiple procedures are performed on one case at one time, ONE base unit is charged plus time</i>		

Billing for time

CODE	TIME	UNIT
T1	Every 15 Minutes or part thereof [For the first TWO hours]	1
T2	Every 10 Minutes of part thereof [From the start of the THIRD hour]	1

Billing for modifying factors

It is strongly recommended that all modifying factors are itemised on your records and the patient's records. In addition, itemisation may be a mandatory requirement of contracts. Modifying units can be added to base units under the following circumstances:

CODE	MODIFYING FACTORS	UNIT
PA1	Telephone Pre-assessment – Simple [<15 minutes]	1
PA2	Telephone Pre-assessment – Complex [>15 minutes]	2
PA3	Face-To-Face Pre-assessment – Prior To Day Of Surgery [Maximum 4 Units]	2 + Time
PA4	Face-To-Face Pre-assessment – On Day Of Surgery – Complex [Complex Assessment + Management Of Significant Issues]	2
PA5	Cancellation of Case on Day of Surgery [After Complex Pre-assessment]	2 [Max]
A1	Age >1, and/but <2, 70-80-year-old	1
A2	Age < 1 year old and/but > 80-year-old	2
AS1	ASA I or II	0
AS3	ASA III	2
AS4	ASA IV	4
ASE	Emergency case	2
OB1	BMI < 35	0
OB2	BMI 35 - 40	1
OB3	BMI > 40	2
OB4	BMI > 50	3
AI1	Awake Intubation - However accomplished	2
P1	Any position that is non-supine, non- lithotomy, or non-lateral <i>IF not already accounted for (It is included in Spine and Neuro Base units)</i>	2

VM1	Vascular monitoring (Art Line, CVL, PA Catheter) NOT EXPECTED of Anaesthesia/Procedure	2
TTE1	Peri-operative Transthoracic Echocardiogram - i.e. Full Report + Formal Report [Accredited Anaesthetist ONLY]	6
TTE2	Non-operative Transthoracic Echocardiogram – i.e. in Emergent Situation To Assess Filling [Accredited Anaesthetist ONLY]	2
PACU1	Recovery Room Care – Provide documentation if >15 minutes being claimed for	As Per T1/T2
EAA1	Elective Additional Anaesthetic Assistance [Discuss with patient + payer pre-surgery if possible]	4
POC 1	ICU Care: Anaesthetist In Hospital Providing Cover per day	25
POC2a	HDU Care: Anaesthetist At Home Providing Cover per day	4-6
POC2b	PLUS: Telephone Call Requiring Significant Management Change	1 [Per Call]
POC2c	PLUS: Face-To-Face Visit Resulting in Significant Management Change	4 + Time
POC3a	Non-Complex Ward/PACU Review – Nerve Catheter/Epidural/IV Fluids/ Analgesia	1+ Time
POC3b	Complex Ward/PACU Review with significant documented management issues	2 + Time
NC1	In situ Plexus/Nerve Catheter patient in hospital	1
NC2	In situ Plexus/Nerve Catheter at hospital discharge [1 Unit/day to maximum of 3 Units]	1

Preoperative consultations for ACC Patients

Information regarding preoperative assessments of ACC patients can be found in the ACC Clinical Services Contract: <https://www.acc.co.nz/assets/contracts/clinical-services-og.pdf>

Specifically on page 10, Table 2 under Anaesthesia Specific Assessments, outlines the codes for pre-operative simple, complex and brief phone consultations that can be claimed for ACC patients.

Table 2

PROCEDURE	CODE	DEFINITION	TREATMENT PROVIDER
Simple Pre-operative Anaesthetic Assessment (Initial)	CS250	Simple Pre-operative Anaesthetic Assessment (Initial) - Electronic consultation (preoperatively only)	Anaesthetist
Complex Pre-operative Anaesthetic Assessment – (Initial)	CS260	Complex Pre-operative Anaesthetic Assessment - (Initial) - Per consultation (preoperatively only)	Anaesthetist
Anaesthetic consultation	CS70	Anaesthetic consultation - Per Call (pre-operatively only)	Anaesthetist

Hypothetical Calculation of Unit Value Using the NZSA RVG

Example: Three-year-old child for grommets, ASA1, procedure time 20 minutes OT to PACU

Base Unit 4

Time unit 2

Modifier 0

Total units 6

Example: Forty-five-year-old woman for laparoscopic hysterectomy, who attends for preoperative assessment prior to admission, ASA2 (asthma and HTN), BMI 37, anxious, procedure time 150 minutes OT to PACU, two post-operative visits for extensive pain management

Preop assess 2

Base unit 6

Time unit 11

Modifying unit 2 (BMI)

Postop visit 2

Total 23

N.B: The pre-operative, modifying and post-operative units are at the discretion of the individual anaesthetist.

Appendix 1: Websites, Links and Documents

1. NZMA statement on Third Party Funding and updated Code of Ethics
https://assets-global.website-files.com/5e332a62c703f653182faf47/5e332a62c703f68f7e2fc657_Third-party-funding-arrangements-2015.pdf

https://assets-global.website-files.com/5e332a62c703f6340a2faf44/5fbd645fe15640fa981fa469_Code%20of%20Ethics%20Redesign%202020%20version%204.pdf
2. Commerce Commission Guide to Setting fees for Health Practitioners on Commerce Commission Website
www.comcom.govt.nz/business
3. HDC site for Patients Code of Rights
<https://www.hdc.org.nz/your-rights/the-code-and-your-rights/>
4. ANZCA PS26 Guidelines on Consent for Anaesthesia and Sedation
[https://www.anzca.edu.au/getattachment/d11e9c7e-0825-458a-af47-7a21ddb588a7/PS26-Statement-on-informed-consent-for-anaesthesia-or-sedation-\(PILOT\)#page=](https://www.anzca.edu.au/getattachment/d11e9c7e-0825-458a-af47-7a21ddb588a7/PS26-Statement-on-informed-consent-for-anaesthesia-or-sedation-(PILOT)#page=)
5. ANZCA PS59 Statement on Roles in Anaesthesia and Perioperative Care
<https://www.anzca.edu.au/getattachment/b954cbdd-ae1-467d-893a-358990d13317/PS59-Statement-on-roles-in-anaesthesia-and-perioperative-care#page=>

Appendix 2: ACC Clinical Services Operational Guidelines

Direct link to above document:

<https://www.acc.co.nz/assets/contracts/clinical-services-og.pdf>